

A SELF-INITIATED PRICING POINT OF VIEW

What a pill is worth

Pricing oral GLP-1s against the injectables that built the market

Two oral GLP-1s reached the US market in seven months. Whether they sell at a premium, at parity, or at a mass-access discount decides who wins the next phase. This is a value-based read on that call, built from the public record.

Public sources throughout. Every modeled price is illustrative, tied to a named public analog.

I intern at Eli Lilly on the science side (ADME data and software). I have no role in, and no visibility into, any company's pricing, commercial, or market-access decisions. Nothing here is internal or under NDA.

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WHY NOW

The market is built. The price call is open.

Injectables turned obesity into one of pharma's largest franchises. Then two oral GLP-1s reached the US market inside seven months, and both priced aggressively from day one.

● Premium, for convenience

Charge for the pill's ease and hold a per-script margin, the way injectables were priced.

● Mass-access, for volume

Price low to reach primary care, cash-pay, Medicare and global buyers, winning on reach instead of margin.

The whole deck is one question: **which model should an oral choose, and why?**

THE MOMENT, IN THREE NUMBERS

~\$100B

Projected obesity-drug market by 2030

Morgan Stanley \$105B · Goldman ~\$95B

2 pills

Oral GLP-1s launched in 7 months

Wegovy pill Dec 2025 · Fodayo Apr 2026

\$149/mo

Starting cash price both launched at

NovoCare · LillyDirect

The tension. Injectables were priced as a premium. The orals arrived priced for volume. This deck asks whether that was right.

THE QUESTION, STATED PRECISELY

Premium, parity, or mass-access discount?

An oral GLP-1 is not obviously worth more or less than an injectable. It is worth something different. Two levers set the answer, and they pull in opposite directions.

01 Value: what a patient or payer will pay

A pill is more convenient but, for now, less effective. Net of those, what is an oral actually worth against the injectable that set the price? That is the ceiling.

Convenience & adherence (+) · Efficacy gap (-)

02 Cost-to-serve: what it costs to make and deliver

A small-molecule pill is far cheaper and more scalable than an injectable peptide. That cost structure decides how far below the value ceiling you can profitably price.

Small-molecule synthesis · No cold chain, no fill-finish

The claim I test. The value ceiling lands near parity, so pricing well below it is a deliberate choice, not a discount forced by weak value.

WHERE THE VALUE COMES FROM

Four drivers, each rooted in the molecule

Pricing an oral against an injectable is a drug-property argument before it is a commercial one. Where the science helps or hurts, the price should follow.

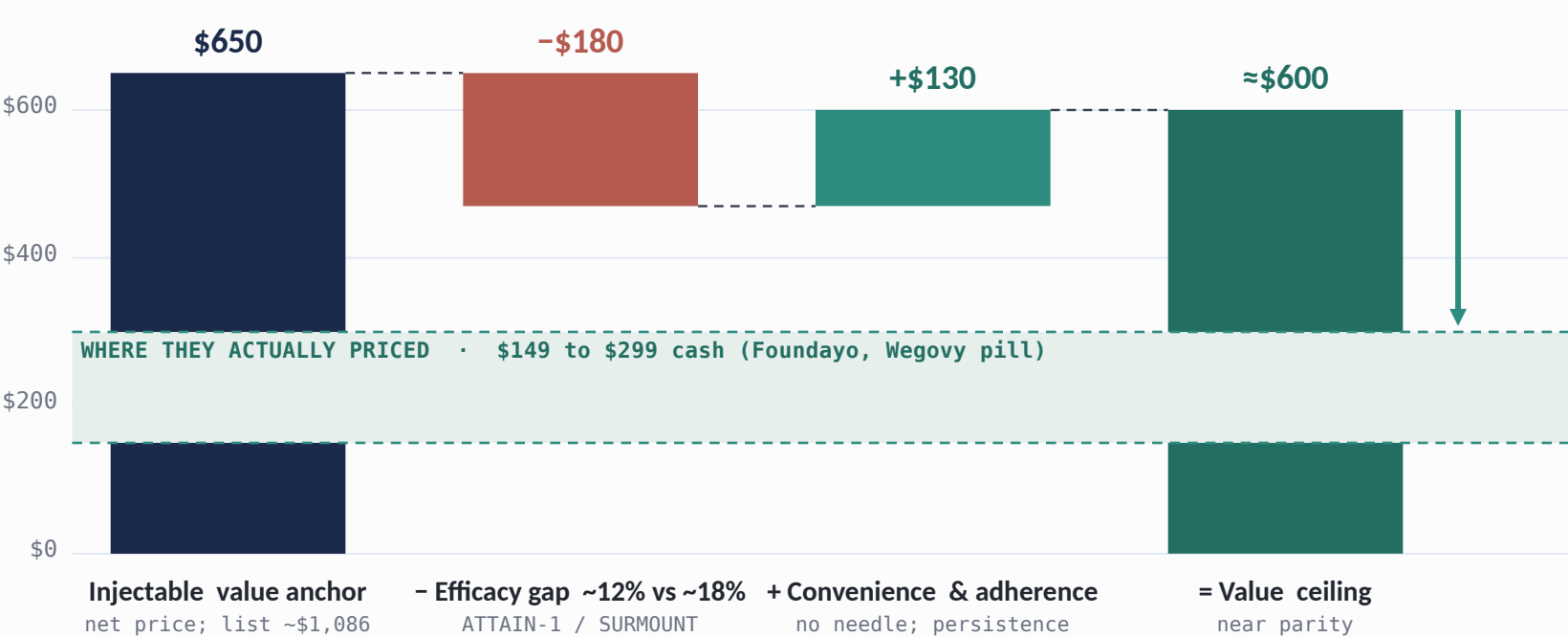
	Efficacy a negative adjustment to value	Oral bioavailability and dosing ceilings cap weight loss. The lead oral small molecule delivers about 12%, against 15 to 21% for injectables.
	Convenience & adherence an uplift, kept modest since the WTP evidence is thin	No needle, no cold chain, a daily pill. About 65% of patients quit within a year, and stopping drives fast regain, so persistence carries real value.
	Access & reach the volume lever, not a per-script premium	Primary-care prescribing, no injection training, global and low-resource distribution. The pill reaches a far larger population.
	Cost-to-serve the margin cushion that funds pricing below the ceiling	Small-molecule synthesis uses roughly 40 to 80 times less material input than peptide manufacturing, and a pill skips fill-finish and cold chain.

The shape of the answer. Two minuses and two pluses. Efficacy pulls the price down; everything else pushes it toward parity and toward volume.

THE VALUE-BASED PRICE MODEL

What is an oral GLP-1 worth, and where should it price?

Illustrative, per patient per month, US. Anchored to a branded-injectable net price, with every step tied to a public analog.



THE GAP IS THE STRATEGY
Value says about **\$600**, near where injectables actually net. Lilly lists Foundayo at **\$649**, right about there.

But almost nobody pays list. Cash runs **\$149 to \$299**. That gap is **a choice**, not weak value: funded by a cheaper cost-to-serve, spent on reach.

Illustrative model. Anchor = injectable net (ICER/SSR est.). Efficacy per NEJM/Lancet readouts; convenience uplift directional. Public launch prices only.

THE MONETIZATION CALL

Price it as a volume play, not a convenience premium

Set the oral at a **modest discount to parity** on value, then use the cost-to-serve advantage to price well below that ceiling. Win on reach, adherence and a lower cost-to-serve, not on per-script margin. The strategic prize is a **bigger market**: primary care, earlier intervention, Medicare and global reach, not a richer script.

Revenue is price times patients. The oral trades a smaller number on the left for a much larger one on the right.

- **Premium** high price, narrow insured and needle-tolerant base
revenue capped by access
- **Mass-access** modest price, primary-care plus cash plus Medicare plus global
revenue scales with reach

THE MARKET ALREADY VOTED

\$149 to \$299

Where both orals actually priced (cash)

Foundayo · Wegovy pill, 2026

~\$50/mo

Medicare copay via the GLP-1 Bridge

CMS, live since Jul 1 2026

So what. Both leaders priced their pills for volume, not convenience. The value model says they had room to charge more, and chose not to. That is the oral's whole strategic logic.

The thesis. An oral GLP-1 is a market-expansion engine. It prices below its value ceiling because the cost structure allows it, and is paid back in volume.

SEGMENTATION, WHO PAYS WHAT

One molecule, four willingness-to-pay curves

“Mass-access” is not one price. The oral’s reach comes from meeting four very different buyers where their value sits.

PAYER / PBM Commercially insured Net price and rebates set the deal. The oral competes on total cost and on adherence, where injectables leak value. compete on net + persistence	THE PATIENT Cash-pay convenience Price-elastic and growing. The \$149 to \$299 DTC channel has become the real reference price for the category. volume; the DTC anchor	CMS / PART D Medicare Opened to obesity this year. The GLP-1 Bridge, live since July, fixes a ~\$50 copay for a large, previously excluded population. access unlock, capped copay	PUBLIC SYSTEMS Global / LMIC No cold chain, no device, just primary-care dispensing. It is the only formulation that scales here, at a low price tier. the largest volume, lowest price
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The flex. Same pill, four prices. Let cost-to-serve headroom fund the low tiers while the insured tier defends net. One product, deliberately spread.

WHAT WOULD BREAK THIS

Four ways the volume thesis fails

I am more confident in the framework than in any single number. These are the pressure-tests I would run first, and two are already underway.

01 The efficacy gap widens

Retatrutide posted 28% weight loss in May 2026. If injectables keep pulling ahead while orals sit near 12%, a “modest discount” quietly becomes a “deep discount,” and the value anchor falls.

02 Payers commoditize the class

IRA negotiation in 2027, the MFN and TrumpRx \$245 injectable price, and Novo’s list cut to \$675 all compress the reference. Reference pricing can erase the parity anchor.

03 A price war, already here

Novo’s pill undercut Lilly, and Lilly matched at ~\$299. A race to the bottom eats the cost-to-serve cushion before volume pays it back.

04 The cost advantage doesn’t hold

It assumes clean small-molecule scale-up. Pfizer’s oral danuglipron died on a liver signal in 2025. Safety, yield, or supply could each erase the margin that funds low pricing.

The open question. Is the oral a market-expansion engine, or just a cheaper way to sell the same script? The next 18 months of volume data decide it.

WHAT A PILL IS WORTH

Price for the market you unlock, not the script you sell

The value ceiling for an oral GLP-1 sits near parity. Its cost structure lets you price far below that, and the payoff is reach, adherence and a larger market. The first two launches priced exactly this way. Whether the volume follows is the question worth watching.

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Built entirely from public information. Every figure is cited on the next slide, and all pricing is illustrative, from public analogs and stated assumptions. I intern at Lilly on the science side and have no role in, or visibility into, any company's pricing. Nothing internal, nothing under NDA.

APPENDIX

Sources and assumptions

All public: figures as reported by companies, the FDA and CMS, peer-reviewed journals (NEJM, Lancet, JAMA), or ICER. Modeled prices are illustrative, tied to the named analogs below. Nothing internal or under NDA.

EFFICACY (WEIGHT LOSS %)

- Orforglipron / Foundayo, ATTAIN-1: 11.2% (ITT), 12.4% on-treatment. NEJM 2025; FDA approval Apr 1 2026
- Oral semaglutide / Wegovy pill, OASIS-4: 13.6 to 16.6%. Novo; FDA Dec 22 2025
- Tirzepatide SURMOUNT-1: 20.9%. Semaglutide STEP 1: 14.9%. NEJM
- CagriSema REDEFINE-1: 22.7%. Retatrutide TRIUMPH-1: 28.3% (May 2026)

PRICING

- Injectable list ~\$1,086 (Zepbound), ~\$1,349 (Wegovy). Lilly, NovoCare
- Injectable net est. ~\$520 to \$700/mo. ICER / SSR Health
- Foundayo list (WAC) \$649/mo; self-pay \$149 / \$199 / \$299 by dose. pricinginfo.lilly.com
- Wegovy pill self-pay \$149/mo. NovoCare
- Medicare GLP-1 Bridge \$50/mo copay, live since Jul 1 2026. CMS

ADHERENCE AND PREFERENCE

- 1-yr discontinuation ~65% (obesity). JAMA Netw Open / Truveta 2025; Prime Therapeutics
- Weight regain on stopping. SURMOUNT-4 (JAMA 2024); STEP 1 extension 2022
- Oral vs injectable preference 76% to 53% once burden is shown. REVISE 2021

COST-TO-SERVE AND MARKET

- Peptide vs small-molecule input, 40 to 80 times. The Medicine Maker (2026)
- Injection-pen cost and cold chain. MSF, Lancet 2026; Lilly \$6.5B API plant (2025)
- Injectable shortages resolved 2024 to 2025. FDA. Market ~\$95 to \$105B by 2030. Goldman, Morgan Stanley

A NOTE ON THE AUTHOR

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